

DIARRHOEA

Give urgent attention to the patient with diarrhoea and any of:

- Thirst, dry mouth, poor skin turgor, sunken eyes, drowsiness/confusion, BP < 90/60, pulse ≥ 100, **dehydration** likely
- If patient has watery diarrhoea (with or without vomiting) and has been in cholera outbreak area in past 5 days, **cholera** likely →47.

Management:

- Give **oral rehydration solution (ORS)** and observe: encourage small frequent sips. Aim for at least 1-2L in first 2 hours. If patient vomits, wait 10 minutes and try again more slowly.
 - If no better after 2 hours, give IV fluids as below and refer same day.
- If unable to drink or BP < 90/60, give **sodium chloride 0.9%** 1L IV rapidly, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens. Refer same day.
- If patient has been in cholera outbreak area in past week, **cholera** likely. Give single dose **ciprofloxacin** 1g orally.

Approach to the patient with diarrhoea not needing urgent attention

- Confirm patient has diarrhoea: ≥ 3 loose stools/day.
- Ask about duration of diarrhoea:

Diarrhoea ≤ 2 weeks

Is there blood in the stool?

No

Yes

Has patient been in cholera outbreak area in past 5 days?

Yes

No

Cholera likely
→47.

Gastroenteritis likely

- Give **loperamide** 4mg initially, then 2mg after each loose stool, up to 12mg/day.
- If vomiting, give **metoclopramide** 10mg 8 hourly as needed for up to 5 days.
- If abdominal cramps are distressing, give **hyoscine butylbromide** 10mg 6 hourly for up to 3 days.
- Advise antibiotics are not needed and to drink lots of fluids.
- If fever with cough/sore throat, consider COVID-19 → 40.
- Advise to return if: blood in stool, diarrhoea worsens or persists > 2 weeks, or patient becomes confused.

Dysentery likely

- Give **ciprofloxacin** 500mg 12 hourly for 3 days.
- If no response within 3 days, refer.

Diarrhoea > 2 weeks

- If blood/mucous in the stool, refer.
- Send stool for ova, cysts and parasites. Indicate on request form if patient has HIV.
- Test for HIV → 110 and manage according to result:

HIV positive

HIV negative or unknown

- Give routine HIV care → 111.
- LPVr can cause ongoing loose stools.
- Review symptoms and stool result in 1 week:

Treat for **giardiasis**: give **metronidazole**³ 2g daily for 3 days.

Isospora belli

Cryptosporidium

Review stool result:

Give **co-trimoxazole** 320/1600mg (4 tablets) 12 hourly for 10 days.

Stool negative

Stool positive

Give **loperamide** 2mg as needed up to 8mg/day.

Treat according to result.

If diarrhoea persists despite treatment, refer for specialist review.

- Advise to increase fluid intake. Advise frequent handwashing, with soap and water, before preparing food/after going to toilet. Wash all surfaces/equipment used in food preparation. Wash and peel all fruit and vegetables. Use only safe/disinfected water for preparing food/drinks/ice. Cook food thoroughly, avoid raw/uncooked food, especially meat and shellfish.
- If repeated episodes of diarrhoea and no access to clean water, refer to health promotion officer/social worker.
- If > 65 years, bed-bound or receiving palliative care, check for solid immobile bulk of stool in rectum. If present, impaction likely: gently remove stool using lubrication. If unsuccessful, refer.
- If patient has a life-limiting illness, also consider giving routine palliative care →170.

¹Rice water stool is cloudy watery diarrhoea with no blood/pus and no faecal odour (may have fishy odour). ²If > 2 hour delay between specimen collection and laboratory processing, discuss with laboratory. ³Advise no alcohol until 24 hours after last dose of metronidazole.